

Partnership				
Low			High	
1	2	3	4	5
Clinician actively assumes the expert role for the majority of the interaction with the client. Collaboration or partnership is absent.	Clinician superficially responds to opportunities to collaborate.	Clinician incorporates client's contributions but does so in a lukewarm or erratic fashion.	Clinician fosters collaboration and power sharing so that client's contributions impact the session in ways that they otherwise would not.	Clinician actively fosters and encourages power sharing in the interaction in such a way that client's contributions substantially influence the nature of the session.

This scale is intended to measure the extent to which the clinician conveys an understanding that expertise and wisdom about change reside mostly within the client. Clinicians high on this scale behave as if the interview is occurring between two equal partners, both of whom have knowledge that might be useful in solving the change under consideration. Clinicians low on the scale assume the expert role for a majority of the interaction and have a high degree of influence in the nature of the interaction.

Verbal Anchors

1. Clinician actively assumes the expert role for the majority of the interaction with the client. Collaboration or partnership is absent.

Examples:

- Explicitly takes the expert role by defining the problem, prescribing the goals, or laying out the plan of action
- Clinician actively forces a particular agenda for the majority of the interaction with the client
- Denies or minimizes client ideas
- Dominates conversation
- Argues when client offers alternative approach
- Often exhibits the righting reflex

2. Clinician superficially responds to opportunities to collaborate.

Examples:

- Clinician rarely surrenders the expert role
- Minimal or superficial querying of client input
- Often sacrifices opportunities for mutual problem solving in favor of supplying knowledge or expertise
- Minimal or superficial responses to client's potential agenda items, knowledge, idea, and /or concerns

- Occasionally may correct the client or refutes what the client has said

3. Clinician incorporates client's contributions but does so in a lukewarm or erratic fashion.

Examples:

- May take advantage of opportunities to collaborate, but does not structure interaction to solicit this
- Misses some opportunities to collaborate when initiated by the client
- The righting reflex is largely absent
- Sacrifices some opportunities for mutual problem solving in favor of supplying knowledge or advice
- Seems to be in a stand-off with the client; not wrestling and not dancing

4. Clinician fosters collaboration and power sharing so that client's contributions impact the session in ways that they otherwise would not.

Examples:

- Some structuring of session to ensure client input
- Searches for agreement on problem definition, agenda setting, and goal setting
- Solicits client views in more than a perfunctory fashion
- Engages client in problem solving or brainstorming
- Does not attempt to educate or direct if client "pushes back" with sustain talk
- Does not insist on resolution unless client is ready

5. Clinician actively fosters and encourages power sharing in the interaction in such a way that client's contributions substantially influence the nature of the session.

Examples:

- Genuinely negotiates the agenda and goals for the session
- Indicates curiosity about client ideas through querying and listening
- Facilitates client evaluation of options and planning
- Explicitly identifies client as the expert and decision maker
- Tempers advice giving and expertise depending on client input
- Clinician favors discussion of client's strengths and resources rather than probing for deficits

Empathy				
Low			High	
1	2	3	4	5
Clinician gives little or no attention to the client's perspective.	Clinician makes sporadic efforts to explore the client's perspective. Clinician's understanding may be inaccurate or may detract from the client's true meaning.	Clinician is actively trying to understand the client's perspective, with modest success.	Clinician makes active and repeated efforts to understand the client's point of view. Shows evidence of accurate understanding of the client's worldview, although mostly limited to explicit content.	Clinician shows evidence of deep understanding of client's point of view, not just for what has been explicitly stated but what the client means but has not yet said.

This scale measures the extent to which the clinician understands or makes an effort to grasp the client's perspective and experience (i.e., how much the clinician attempts to "try on" what the client feels or thinks). Empathy should not be confused with sympathy, warmth, acceptance, genuineness, support, or client advocacy; these are independent of the Empathy rating. Reflective listening is an important part of this characteristic, but this global rating is intended to capture *all efforts* that the clinician makes to understand the client's perspective and convey that understanding to the client.

Clinicians high on the Empathy scale show evidence of understanding the client's worldview in a variety of ways including complex reflections that seem to anticipate what clients mean but have not said, insightful questions based on previous listening and accurate appreciation for the client's emotional state. Clinicians low on the Empathy scale do not appear interested in the client's viewpoint.

Verbal Anchors

1. Clinician gives little or no attention to the client's perspective.

Examples:

- Asking only information-seeking questions
- Probing for factual information with no attempt to understand the client's perspective

2. Clinician makes sporadic efforts to explore the client's perspective. Clinician's understanding may be inaccurate or may detract from the client's true meaning.

Examples:

- Offers reflections but they often misinterpret what the client had said
- Displays shallow attempts to understand the client

3. Clinician is actively trying to understand the client's perspective, with modest success.

Examples:

- May offer a few accurate reflections, but may miss the client's point
- Makes an attempt to grasp the client's meaning throughout the session

4. Clinician makes active and repeated efforts to understand the client's point of view. Shows evidence of accurate understanding of the client's worldview, although mostly limited to explicit content.

Examples:

- Conveys interest in the client's perspective or situation
- Offers accurate reflections of what the *client has said already*
- Effectively communicates understanding of the client's viewpoint
- Expresses that the client's concerns or experiences are normal or similar to others'

5. Clinician shows evidence of deep understanding of client's point of view, not just for what has been explicitly stated but what the client means and has not said.

Examples:

- Effectively communicates an understanding of the client *beyond* what the client says in session
- Shows great interest in client's perspective or situation
- Attempts to "put self in client's shoes"
- Often encourages client to elaborate, beyond what is necessary to merely follow the story
- Uses many accurate complex reflections